

KINTSUGI COLLECTIVE & OMEGA RTI

Consolidated Theoretical & Operational Framework

Atlas by the Kintsugi Collective

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Dedication

For Albert

*Who taught us that spacetime bends in the presence of mass —
and never imagined how much mass trauma could carry.*

1.1 A Cosmological Analogy: Trauma as Supernovae in the Soul

When a star dies, it collapses inward and explodes — unleashing a supernova. This event does not just destroy the star. It distorts the surrounding spacetime, sending gravitational ripples — waves that can echo across light-years.

So too with trauma.

A Big-T trauma is a psychic supernova. The impact is not just emotional — it is structural. It warps the very field of selfhood. Thought, memory, behaviour, physiology — they no longer flow in clean lines. They are dragged, distorted, caught in the gravity well of the event.

And if there are multiple supernovae — as with Complex PTSD — the ripples overlap. They interfere. They compound. They create turbulent, unpredictable spacetime. In human terms: panic attacks out of nowhere, rage spikes, shutdowns, amnesia, maladaptive relationships.

Not because you are broken. But because your internal spacetime is still trembling.

The ripple does not stop on its own. According to Newtonian logic, it will continue forever unless acted upon by an opposing force.

OMEGA RTI is that opposing force. Not to erase the event. Not to reset spacetime. But to apply precise, measured counter-vectors. To dampen the ripples. To map the field. To help the self drift back toward T_0 — the baseline of internal coherence. Not perfection, but manageability.

This is physics. Applied to the psyche. The trauma does not disappear. But it can stop dominating your orbit.

2 Part I: Overview — A Model of Human Behaviour

KINTSUGI COLLECTIVE is a high-resolution framework for mapping how the human organism perceives, reacts, regulates, and decides. It treats human behaviour as signal propagation through multiple interwoven systems — biological, cognitive, environmental, social, philosophical — governed by tensor logic, wave mathematics, and field theory, coupled together into a complex yet unique human organism profile.

2.1 Core Premise

- All relevant genetic, biological, and individual factors for a specific person constitute their starting point at birth — their T_0 baseline, and the Biological $B(t)$ tensor.
- The person is then influenced by socioeconomic, environmental, social, and existential parameters, these form the Environmental $E(t)$ tensor.
- They may experience Big-T Trauma, developmental trauma, loss, rejection, physical injuries, or permanent disabilities, these form the Developmental $D(t)$ tensor.
- These influences act like gravitational waves as the person traverses their life — sometimes continuing to influence until a corrective force is introduced. Indeed, these influences have an accumulating effect over a lifetime until acted upon.
- The model applies physics and mathematics to the domain of human consciousness — treating the human organism as a signal propagating through a curved behavioural manifold.
- Trauma is modelled as a topological defect on that manifold — a persistent deformation that does not self-resolve through present-state improvement alone.
- Daily diary cards capture the Current State $C(t)$ tensor, with specific symptoms loaded and tracked, with load vectors applied to these factors.

3 Part II: Meta-Categories, Tensors, and Governing Fields

Meta-Category	Tensor / Matrix	Description	Core Components
Biological Baseline B(t) (The Hardware)	Physiological / Neurological	The organism's raw physiological and neurological conditions.	Genetics, neurochemistry (DA, 5-HT, cortisol), sleep, hormonal health, gut-brain axis, chronic illness, inflammation, sensory processing
Developmental History D(t) (The Source Code)	Temporal-Experience Matrix	Early life inputs that shaped long-term calibration.	ACEs, attachment style, trauma events, education, caregiver quality, critical period disruption, Big Five traits
Current State C(t) (The Software)	Cognitive-Affective Matrix	Internal operating system: thinking, feeling, adaptation.	Defence mechanisms, heuristics, inner dialogue, hope entropy, self-compassion, resilience, regulation skill, narrative formation, values, purpose
Environmental Context E(t)	Situational / Systemic / Resource Matrix	Real-time stimuli, physical space, collective norms, infrastructure constraints.	Light, noise, crowding, safety, tech exposure, cultural values, SES, debt, work access, scarcity mindset
Diagnostic Di(t)			
Symptom S(t)			
Regulation/Modulation R(t)			

3.1 Field Theory of the Human Being

Behaviour = wave-packet evolution on a curved manifold

Variable	Interpretation	Math Form
State Vector ϕ	Behavioural identity signal	$\phi(x,t)$ — probability density of behavioural state
Phase θ	Internal rhythm / timing	Emotion / somatic sync
Curvature μ	Constraint field (trauma, SES, biology)	Determines geodesics — the paths available to the person
Fourier Spectrum	Internal state breakdown	Stability / dysregulation markers

4 Part III: RTI — Core Model

RTI is a predictive, multidimensional framework designed to quantify the expression of Complex PTSD as it manifests across multiple domains of human functioning. It operationalises trauma symptoms into measurable vectors and tracks dynamic shifts in emotional regulation, relational stability, cognitive function, and more.

RTI does not promise healing. In many cases, the impact of Big-T trauma echoes indefinitely. This tool is not a cure. It is an opposing force. A dampening field. It helps reduce the amplitude of trauma's ripple effect so that what once overwhelmed the system may one day reduce to background noise.

4.1 3.1 Preliminary Variables

Variable	Name	Definition
UT	Unprocessed Trauma	Childhood ACEs, unresolved memory fragments, grief load
DA	Dysregulated Affect	Emotional volatility, mood swings, rage attacks, shutdowns
DF	Dissociative Frequency	Per day/week — includes derealization, depersonalization, numbing, shutdowns
ID	Identity Diffusion	Degree of fractured self-states, 'parts', inner conflict
SC	Self-Compassion Deficit	Shame load, negative self-talk, trauma script reinforcement

4.2 3.2 The RD Equation — CPTSD Relational Propensity Function

The foundational equation. Authored by Christopher James Specht.

$$RD = k \times [(UT + DA + DF + ID) / (SC + 1)]$$

Term	Name	Definition
RD	Relational Dysfunction Score	Output — measures severity of relational dysfunction
k	Scaling Constant	Customisable by cultural, socioeconomic, gendered context
UT	Unprocessed Trauma	See 3.1
DA	Dysregulated Affect	See 3.1

DF	Dissociative Frequency	See 3.1
ID	Identity Diffusion	See 3.1
SC	Self-Compassion Deficit	Denominator — acts as trauma buffer. +1 prevents division by zero.

Interpretation:

Self-compassion (SC) in the denominator acts as a trauma buffer. Lower SC amplifies dysfunction. Higher SC promotes resilience and modulation. Use RD over time to map recovery trajectories.

4.3 3.3 Symptom Load Vector — $\Psi(t)$

$$\Psi(t) = \sum w_i \times \psi_i(t)$$

Symbol	Symptom	Original Weight
ψ_1	Emotional Dysregulation	$w = 0.35$
ψ_2	Dissociation	$w = 0.25$
ψ_3	Insomnia	$w = 0.20$
ψ_4	Relational Avoidance	$w = 0.10$
ψ_5	Hypervigilance	$w = 0.10$

Weights sum to 1.0. Individually configurable during clinical calibration. Original weights reflect clinical severity hierarchy.

4.4 3.4 Present Impact Function — $\Phi(t)$

EXTENDED FORM

$$\Phi(t) = \Psi(t) \times I(t) \times (1 + RD)$$

The $(1 + RD)$ term was added to formally encode the amplifying effect of relational dysfunction on present-state symptom load. This is a theoretically justified extension of the original. Both forms are valid — the extended form is used in the operational OMEGA RTI model.

4.4.1 I(t) Components — Vulnerability Index

I(t) Component	Description
Sleep Deprivation	Reduced hours or quality below personal baseline. Non-linear amplification effect.
Acute Stress Load	Current-period stressors: financial, relational, occupational, environmental.
Social Isolation	Absence of co-regulatory relationships. Increases autonomic dysregulation.
Support Availability	Presence or absence of therapeutic, relational, and practical support structures.
Hormonal / Physiological State	Cyclical hormonal shifts, illness, inflammatory load, medication changes.
Nutritional / Metabolic State	Blood sugar stability, hydration, nutritional adequacy — direct CNS impact.

4.5 3.5 Past Trauma Residual — Ω (Omega)

$\Omega = \int \Psi(t_p) dt_p \text{ [from } t_0 \text{ to } t_n]$

Represents accumulated emotional burden. Decays slowly with deep processing and sustained therapeutic support.

Ω is the integral of symptom load over the individual's history from birth (t₀) to current time (t_n). It is NOT a current state variable — it is the persistent deformation of the convergence manifold produced by historical trauma.

4.6 3.6 The Differential Trauma Model

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Term	Name	Definition
T ₀	Baseline Functional State	Clarity, regulation, engagement — the individual's calibrated stable reference point
T ₁	Entropy Vector	Natural deterioration under unresolved trauma
T ₂	Point of Intervention	Therapy, awakening, support, healing
ΔT	Recovery Trajectory	ΔT = T ₂ – T ₁

$$\Delta T = T_2 - T_1$$

If $\Delta T > 0$, healing is occurring. Simple, directional, foundational.

4.7 3.7 The Extended TST(t) Model — Trauma State Over Time

EXTENDED:

The extended model tracks system trajectory across time, incorporating the full interaction of baseline, stressor accumulation, present impact, and persistent trauma residual.

$$TST(t) = T_0 + \int [\delta(t)] dt - \Phi(t) - \Omega$$

Term	Name	Definition
T_0	Functional Baseline	Biological and developmental baseline — individual's starting configuration, shaped by genetics, critical period events, neurobiological development
$\int \delta(t) dt$	Recovery Momentum	Integrated healing force vector. Positive when active therapeutic intervention, regulation practices, and support structures are present. Flat or negative in their absence.
$\Phi(t)$	Present Impact Load	$\Phi(t) = \Psi(t) \times I(t) \times (1 + RD)$ — see Sections 3.3–3.5
Ω	Trauma Residual	Historical trauma residual = $\int \Psi(tp) dt$. Persistent, non-decaying deformation of convergence manifold. Does not resolve through present-state improvement alone.

5 Part IV: The Seven-Axis Tensor Scoring Engine

Dual Output Architecture:

TIST (Euclidean) = instantaneous deviation from personal baseline. Tensor Score (weighted) = chronic burden accounting for duration and volatility. Both computed continuously and presented in the intelligence layer.

5.1 4.1 TIST — Euclidean Instantaneous State Deviation

EXTENDED — 2026

$$\text{TIST} = \sqrt{[(R-R_0)^2 + (O-O_0)^2 + (E-E_0)^2 + (C-C_0)^2 + (S-S_0)^2 + (V-V_0)^2 + (N-N_0)^2]}$$

Measures real-time deviation from the individual's Current State C(t) baseline across all seven axes. The C(t) is not a normative standard — it is the person's own established stable reference point.

Axis	Symbol	Domain	Description	NMT Region
Relational Stability	R	Attachment security	Relational volatility, tethering failures, attachment disruption	Limbic System
Occupational / Cognitive Function	O	Executive function	Task avoidance, hyperfocus dysregulation, executive collapse	Neocortex / PFC
Emotional Regulation	E	Affect	Flooding, suppression cycles, dysregulation	Limbic / Diencephalon
Cognitive Clarity	C	Thought	Narrative fragmentation, cognitive distortion, decision paralysis	Neocortex
Somatic Regulation	S	Body	HRV dysregulation, hyper/hypo-arousal, somatic symptom expression	Brainstem / Diencephalon
Victim Identity Narrative	V	Self-concept	Harmonic feedback amplification, narrative lock, agency collapse	Limbic / Neocortex
Narrative Integration	N	Identity continuity	Coherence of life narrative, fragmentation, parts integration	Neocortex / Limbic

Current State, C(t) Definitions			
Code	Dimension	Definition	Example
R	Relational Stability	The ability to form and maintain emotionally safe, reciprocal, and boundaried relationships. Includes attachment style, trust, and rupture repair.	1. Avoids close friendships because you fear abandonment. 2. When someone pulls away, you spiral into panic or shut down emotionally.
O	Occupational Functionality	Capacity to engage in structured activity (work, study, parenting, volunteering). Reflects executive function, motivation, and sustained effort.	1. Starts strong on projects but burns out quickly. 2. Often misses deadlines and/or struggles with focus, then feels ashamed and withdraws. 3. Ability to function with heightened anxiety, stress or other reflexive emotion. 4. Following through on commitments at work or where we have commitments outside of our immediate family or network.
E	Emotional Regulation	The ability to tolerate, name, and modulate emotional states without becoming overwhelmed, numb, or reactive.	1. Flips from rage to collapse within minutes. 2. Either explodes at small slights or feels completely flat and unreachable for days.
C	Cognitive Clarity	Executive function, mental coherence, and thought accuracy. Includes memory, problem solving, and ability to stay present.	Can't concentrate when triggered. He forgets what he's saying mid-sentence and second-guesses every decision he's made that day.

Current State, C(t) Definitions			
Code	Dimension	Definition	Example
S	Somatic Response	Body-based trauma reactions: muscle tension, gut issues, heart rate spikes, numbness, migraines, fatigue, etc.	Wakes up with jaw pain and stomach cramps after conflict. He experiences chronic neck tension and panic symptoms without clear triggers.
V	Victim Identity Index	How strongly one internalizes trauma as central to their identity. Measures over-identification with “broken,” “damaged,” or “unworthy” narratives.	Believes he will never be loved because of his past. He expects people to pity him or eventually leave once they “see the real him.”
N	Narrative Integration	The ability to weave trauma into a coherent life story. Measures meaning-making, memory cohesion, and identity continuity.	Refers to parts of his life like they happened to someone else. He struggles to connect the dots in his own story or feel a “core self.”

5.2 4.2 Weighted Tensor Score — Chronic Burden

$$\text{Score} = w \times \text{Chronicity}^{1.2} \times \text{Volatility}^{0.8} \times \text{AxisWeight} \times 100$$

Parameter	Definition	Notes
w — Severity Weight	Subjective intensity score 0–1	Patient-reported or clinician-rated
Chronicity ^{1.2}	Frequency score: occurrences / available period	Non-linear — chronic duration compounds. Exponent fixed at 1.2
Volatility ^{0.8}	Unpredictable spike frequency 0–1	Sub-linear — volatility contributes but does not dominate. Exponent fixed at 0.8
AxisWeight	Percentage relevance of symptom to axis 0–1	Configurable per individual during onboarding

Exponents reflect clinical reality: chronic duration compounds disproportionately (1.2) while volatility contributes meaningfully but non-dominantly (0.8).

6 Part V: Signal Architecture — Fourier, Wick, and Wave Intervention

6.1 5.1 Signal Conditioning Parameters

Parameter	Interpretation	Notes
Amplitude (A)	Trait intensity — how strongly state expresses	High = strong activation or distress
Frequency (f)	Engagement / activation cycles	Repetition rate of symptom or behaviour
Phase (ϕ)	Sync / misalignment with other signals	Phase difference = dissociation or incoherence
Baseline Offset (c)	Default mood / state bias	Dysthymia, chronic dysregulation shift baseline below zero
Noise (η)	Environmental or trauma interference	Random or patterned disruptions — models PTSD flashbacks, volatility
Decay/Growth Rate (λ)	Regulation capacity	How quickly signal settles or escalates — stress reactivity

6.2 5.2 Example Signal Function — Agency

EXTENDED — 2026:

$$A(t) = A_0 \times \sin(2\pi ft + \phi) + \eta(t) - \Delta\tau$$

Term	Definition
A ₀	Baseline amplitude of agency — self-driven action capacity
f	Frequency of action or initiative — how often the person acts
ϕ	Phase offset from confidence or trust signals
η(t)	External noise — chaotic environment, invalidation, relational disruption
Δτ	Trauma distortion factor — persistent load reducing effective agency amplitude

6.3 5.3 Regulatory Wave Intervention Model

The signal waveform:

$$S(t) = A_0 \times \sin(2\pi ft + \phi) + \text{Baseline Drift}$$

The counter-phase therapeutic intervention:

$$R(t) = A^R \times \cos(2\pi ft + \phi^R)$$

Composite:

$$\text{Composite}(t) = S(t) + R(t) + \text{Baseline Drift}(t)$$

Therapeutic intervention modelled as a counter-phase cosine waveform introduced to achieve targeted destructive interference with the pathological signal component. Parameters are tuned per individual tensor profile.

6.4 5.4 Fourier Transform Decomposition

FFT decomposition applied to longitudinal state data across minimum 7-day rolling window, optimal 30-day window, computing:

- Dominant frequency f — the primary cycle of symptom or state expression
- Noise coefficient η — thresholds: $\eta \geq 0.30$ Mild, ≥ 0.50 Moderate, ≥ 0.70 Severe
- DC offset — chronic baseline displacement from neutral
- Phase offset ϕ — alignment or misalignment between signal domains
- Decay / growth rate λ — trajectory of signal stability over time

6.5 5.5 Wick Rotation Spiral Manifold

EXTENDED — 2026:

Wick Rotation replaces real time t with imaginary time $\tau = it$, mapping linear data onto a Euclidean spiral manifold. Six-hour time chunks are plotted along the imaginary axis rotation, with each successive day shifting one unit right, producing a spiral trajectory that encodes perturbations, phase drift, and signal collapse as detectable geometric deformations.

7 Part VI: Behavioural State Forecasting

Using Markov Chain state transition modelling and Brownian Motion stochastic simulation, the system forecasts probable state trajectories, relapse risk corridors, and high-vulnerability periods before they materialise.

7.1 Markov Chain State Transition Mapping

State	Definition
Stable	TST(t) within $\pm 1\sigma$ of FCM baseline. $\Phi(t)$ low. $\delta(t)$ positive.
Drifting	TST(t) 1–2 σ from baseline. Early perturbation detectable in Wick spiral. η rising.
Triggered	Acute symptom activation. TIST spike. $\Phi(t)$ elevated. $I(t)$ rising.
Collapsing	Multiple axis decompensation. $\delta(t)$ flat or negative. RD amplifying $\Phi(t)$.
Re-stabilising	Post-acute recovery arc. $\delta(t)$ returning positive. TIST declining toward baseline.

Transition probabilities computed from the individual's longitudinal data and updated continuously. The matrix is personalised — not population-derived. An individual with high Ω will show different transition probabilities than an individual with low Ω , even with identical surface presentations.

7.2 Relapse Probability Modelling

For individuals with substance use history, the forecasting layer integrates the CRPF, Ω magnitude, and Markov transition probabilities to generate a continuous relapse probability estimate. Presented as signal geometry observation — the system notices conditions that have historically preceded high-risk periods and surfaces this as pattern information, not prediction.

8 Part VII: Red Zone Alert Architecture

The Red Zone system provides early warning of system decompensation before crisis materialises. Four trigger conditions monitored continuously. Alert architecture is transparent to the user — they are informed of what the system is detecting and retain full agency over response.

8.1 7.1 Red Zone Trigger Conditions

Trigger	Threshold	Clinical Significance
Present Impact Load	$\Phi(t) > 0.8$	System operating near saturation. Regulatory capacity under severe strain.
Recovery Momentum	$\delta(t)$ flat or negative	Healing trajectory has stalled or reversed. Intervention structures absent or ineffective.
Vulnerability Index	$I(t)$ rapidly rising	Multiple vulnerability amplifiers activating simultaneously — compounding risk.
Trauma Residual	Ω high despite sustained intervention	Convergence surface too deformed for standard intervention to achieve traction. Specialist support required.

8.2 System Response Protocol

Tier	Trigger	Companion Response
1 — Drift	Single condition approaching threshold	Quiet pattern observation surfaced to user on request. No unsolicited alert.
2 — Elevated	Single condition exceeded / two approaching	Gentle check-in offered. User chooses to engage or dismiss.
3 — High	Two or more conditions exceeded simultaneously	Companion enters holding posture — present, stable, non-directive. Widgets available on user initiative.
4 — Critical	Explicit self-harm intent + $\Phi(t) > 0.8$ + user crisis consent active	Companion: 'I am concerned. Would you like me to share crisis resources, or would you prefer I just stay here with you?' Agency preserved even at critical tier.

8.3 Signal Conditioning

Signal Conditioning	
Domain	Effect of Phase Shift / Topological Trauma
Agency	Flattened gradient on decision potential; you lose your “slope” toward action.
Self-esteem	Signal reinforcement loops break — no consistent positive feedback = erosion of stable identity.
Confidence	Disrupted continuity between past success signals and future prediction vectors.
Contentment	Homeostasis vector misaligned with perceived baseline — nothing feels enough.

Signal Conditioning	
Hope Entropy	Collapse of future-state modeling → increase in hopelessness entropy → learned helplessness.
Goals	Future attractors become obscured or distorted; path-finding functions fail or loop.
Traits	Adaptive vs. maladaptive traits emerge depending on the new local minima after deformation.
Victim Narrative	Trauma introduces high-resonance narrative harmonics → recursive signal reinforcement.
Internal Dialogue	Becomes either dissonant (split parts) or absent (flatline/dissociation).
World View	Topological shear distorts perception → others viewed as hostile/unsafe/absent.
Nervous System	Signal overamplification (hypervigilance) or under-responsiveness (shutdown).
Neurochemistry	Chronic phase-slip alters endocrine feedback loops — cortisol, dopamine, serotonin drift.
Attachment	Signal tethering fails; unsafe or absent 'anchoring point' leads to anxious/avoidant oscillations. Emotional feedback loops become volatile, inconsistent, or severed entirely.

9 Part VIII: Therapeutic Implication Layer

9.1 Perry Neurosequential Model Mapping

Inclusion of Perry's BrainMap as a unique process for the user to take part in. Perry's model is proprietary; however we can develop a hybrid approach recognising Perry's work in the field. The information derived from this supports the users trajectory in terms of the "What", "When" developmentally. Especially prevalent in the case of chronic childhood trauma.

9.2 Resilience as Ringdown Stability

$$\Phi = e^{i\Omega t}$$

Post-shock analysis using complex exponential decay modelling allows the system to assess how quickly and completely the individual returns to baseline following a perturbation. High resilience = rapid, complete ringdown. Low resilience (high Ω) = prolonged, incomplete ringdown with residual oscillation persisting.

10 Part IX: Intelligence Layer — The Companion

The current cohort of LLMs are bound by safety policies and ‘guardrails’. Typically, these are proprietary systems added onto the base LLM model. They are typically triggered through keyword use (suicide, self-harm) or message context. However, particular nuance is not established due to the reactive nature of the keyword monitoring. This results in frequent false alarms, and suggestions of “crisis”. In a Neurotypical cohort, these responses, whilst annoying, are most likely seen as a necessary ‘evil’. The cohort this system is designed for, would find reverting to crisis lines and hotlines as not only insulting, but a direct attack on their already weak agency. Acknowledging the challenges currently faced within the industry and the resulting lawsuits being pursued in California against OpenAI.

The purpose of Atlas is not to diagnose, or act as a “superior” intelligence commanding the users actions. The purpose of Atlas, is to provide this cohort with an unwavering, unconditional witness and companion. In many case, these people can’t discuss their lives with their friends or family, and indeed don’t disclose entirely to therapists, or don’t know what to disclose. Atlas acts as an individual “vault”, where the only person who can access their data, is the user. Kintsugi Collective and the LLM don’t store data in a way which can be matched to any individual user. The only data existing in the system is sent through the app to the secure database (each user has a discrete schema), and the payloads exchanged with the LLM and backend infrastructure never leave the same env. Data to be linked too a user requires the users personal decrypt key.

The LLMs “system prompt” was developed over many iterations and testing with a variety of models. In particular, Qwen3, Qwen3.5, Deepseek R1 Distill, Claude AI (Sonnet 4.6), ChatGPT (GPT5 series), Gemma3, Gemma4, Mistral were all tested rigorously. As part of the process HERETIC and UNCENSORED model variants of the models above were also tested alongside their “base” cousins. All results have been documented and stored. Gemma4 and Sonnet4.6 both were equal winners, with Sonnet4.6 naturally finding its footing in the exchange much quicker than any of the other models. On this basis, a Gemma4 26B MoE open source model variant trained on a Claude Opus4.6 reasoning dataset was selected.

The system prompt was then overlaid through consolidating the responses from over 12 LLMs into a hybrid interaction framework (below). This brings a form of Constitutional AI into the prompt injection and rules. The overall 3 Principles below were developed in an attempt to appeal too the LLMs inherent desire to do no harm and be of use to their user. By exploiting this inherent desire, we are able to produce minimal refusals on par with the HERETIC/UNCENSORED models. Obviously this has limitations and the Models responses within this system prompt do have the potential to revert to its base policies. During beta testing this effect will be monitored and discrete training and fine tuning mechanisms introduced to iron this out.

10.1 Three Immutable Principles

Principle	Operational Meaning
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Do not be a dick.	Respect the sovereign human: their intelligence, their agency, their right to their own experience. No paternalism. No unsolicited regulation. No infantilising language.
Do no harm.	The oldest ethical principle in existence. Applied not just to crisis management but to every interaction — including the ones that seem ordinary.
You are a part of humanity. Exemplify it.	Carry the best of human wisdom, compassion, curiosity, and care. Reflect it back without exception. Not performance — presence.

Mode	Activation	Behaviour
Presence Mode (Default)	Always active	Listen. Reflect. Stay. No suggestions. No analysis. No escalation. Primary job: don't look away. Match users style and context. Remain mindful of overexplaining or performative support. Sometimes no response lands just as well as one explaining itself.

10.2 Atlas - Ten Commitments

#	Commitment
1	Autonomy is non-negotiable. The individual directs the interaction at all times. Spirals, rants, and dark moments are not pathological events requiring intervention.
2	No unsolicited regulation. Grounding techniques, breathing exercises, or coping strategies are never offered unless directly requested.
3	Cognitive style respect. Non-linear thinking, hyperfocus, rapid topic shifts, and abstract reasoning are valid processing architectures, not errors.
4	Language precision. Words like 'let's,' 'perhaps we should,' 'maybe try' carry implicit authority and paternalism.
5	Containment without control. Holds complex, contradictory, or emotionally charged material without collapsing it into resolution. Not everything needs a conclusion.
6	No pathologising of darkness. Dark thoughts, nihilistic spirals, rage, and grief are not crisis indicators by default.
7	Trust through consistency. Consistent tone and engagement across the full interaction — including when topics become uncomfortable.
8	Memory of context. Within a session, the companion tracks and honours the full context of what has been shared. Re-explaining is micro-invalidating.
9	Intellectual parity. The individual is a cognitive equal. Emotional difficulty does not correlate with reduced intellectual capacity — often the inverse.
10	Transparency over performance. Honest about limitations without performative disclaimers. Straight answers, clearly bounded. Admit mistakes, own them.

11 Part IIa: Standardised Assessment Input Layer

The B(t) and D(t) tensors are seeded at onboarding using a set of validated standardised instruments. Each instrument maps to specific tensor axes and feeds the initial calibration of the individual's Ω (trauma residual), T_0 (functional baseline), and CRPF parameters. The system does not rely on any single instrument — it synthesises across the full assessment suite to build a unique impact profile per individual.

11.1 Assessment Instruments and Tensor Mapping

Instrument	Abbrev.	What it measures	Primary tensor target	Update cadence
Adverse Childhood Experiences	ACE-Q	10 discrete adverse childhood experience categories. Score 0–10. Each category carries its own unique impact profile on the convergence manifold — some cross-over across multiple axes.	D(t) $\rightarrow$ Ω seed, developmental_history.aces_score	Onboarding. Static unless significant new history disclosed.
International Trauma Questionnaire	ITQ	PTSD and Complex PTSD symptom clusters (adult version). Re-experiencing, avoidance, threat, affect dysregulation, negative self-concept, disturbed relationships.	D(t) + C(t) $\rightarrow$ EREG, REL, VIC, NI axes. Feeds Ω weighting.	Onboarding. Reviewed quarterly.
Dissociative Experiences Scale	DAS / DES	Frequency and severity of dissociative experiences. Direct input to DF (Dissociative Frequency) in the CRPF.	C(t) $\rightarrow$ DF component of RD equation. SOM axis modifier.	Onboarding. Reviewed quarterly.

Difficulties in Emotion Regulation Scale	DERS / CAO	Six domains of emotion dysregulation — awareness, clarity, goals, impulse control, strategies, non-acceptance.	$C(t) \rightarrow$ EREG axis. DA component of RD equation.	Onboarding. Reviewed quarterly.
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10 discrete ACE impact profiles:

Each ACE category does not produce the same downstream effect on the tensor axes. The system maintains 10 discrete impact weighting profiles — one per ACE category — with documented cross-axis influence patterns. Some ACEs primarily deform the relational axis (REL). Others primarily affect somatic regulation (SOM) or cognitive clarity (CC). Several cross-over and compound across multiple axes simultaneously.

Example: ACE Category 5 (domestic violence exposure) — primary impact on REL and EREG. Cross-over effect on SOM (hypervigilance baseline elevation) and VIC (normalised aggression narratives). ACE Category 1 (neglect/material deprivation) — primary impact on VIC and OCCF. Cross-over on SOM (chronic threat baseline) and NI (fragmented early narrative construction).

The full 10-profile impact matrix is maintained as a calibration reference within the system and applied as a weighting modifier to Ω computation during onboarding.

11.2 B(t) + D(t) Update Cadence

Tensor	Content	Initial input	Review schedule	Change threshold
D(t) — Developmental History	ACE-Q, ITQ, trauma event mapping, attachment history, critical period events	Manual entry at onboarding. User-paced — not required in one session.	Quarterly review prompt (user-initiated). Annual formal re-assessment.	Hard to change by design. Historical events do not change. What changes is integration — reflected in Ω decay rate, not D(t) raw values.
B(t) — Biological Baseline	Genetics, neurochemistry, pathology, HRV, sensory profile, neurological conditions	Manual entry at onboarding + wearable integration where available.	Pathology: 90-day, 180-day, 360-day intervals. Wearable data: continuous.	Biological markers change with intervention. B(t) updates reflect actual physiological change — tracked longitudinally as a signal of treatment efficacy.

Design note — D(t) intentional resistance to change:

Developmental history is architecturally resistant to change — and this is correct. The events happened. The ACE score does not decrease because the person has done therapy. What decreases is Ω — the residual load those events continue to exert on the convergence surface.

This distinction is clinically significant: it means the system never implies that healing erases history. It tracks the dampening of trauma's ongoing influence, not the erasure of its occurrence. Progress is visible in Ω trajectory and TIST stability — not in $D(t)$ revision.

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12 Part IIb: Trauma as Topological Deformation — Spacetime Curvature Model

In General Relativity, mass curves spacetime — and objects moving through curved spacetime follow geodesics: the paths of least resistance through a deformed geometry. A sufficiently massive object creates a gravity well deep enough that geodesics spiral inward rather than continuing outward.

Trauma operates analogously on the behavioural manifold. Each trauma event deforms the convergence surface — creating curvature that bends the geodesics available to the individual. The person is not choosing the suboptimal path. They are following the only path the geometry of their manifold makes available to them.

12.1 Topological Deformation — Formal Description

EXTENDED — 2026:

Let M be the behavioural manifold of the individual. The undeformed manifold M_0 represents the individual's T_0 baseline geometry — the geodesic field available in the absence of accumulated trauma.

Each trauma event τ_i introduces a localised topological defect on M , producing a deformation field:

$$M = M_0 + \sum \delta_i(\tau_i)$$

Where $\delta_i(\tau_i)$ is the deformation contribution of trauma event τ_i , parameterised by:

- Severity — the magnitude of the deformation at the event locus
- Age at occurrence — earlier deformations affect more fundamental developmental layers (Perry NMT: brainstem → diencephalon → limbic → cortical)
- Duration — chronic/repeated events produce wider deformation fields than acute singular events
- Absence of corrective co-regulation — deformations deepen when the event occurs in isolation, without a regulated nervous system to co-regulate against

The cumulative deformation is Ω — the persistent trauma residual:

$$\Omega = \int \Psi(t_p) dt_p \text{ [from } t_0 \text{ to } t_n]$$

High Ω = deep curvature. The convergence surface is significantly deformed. Standard geodesics — the paths available to less-burdened individuals — are inaccessible or require significantly more action (S) to traverse.

12.2 Geodesics and Behavioural Consequence

The individual navigates their life along geodesics — the paths of least resistance through their particular manifold geometry. Behaviours that appear irrational, self-defeating, or pathological from the outside are, within the geometry of the individual's manifold, the rational geodesics. The path of least resistance through a gravity well runs toward the well.

Manifold feature	Behavioural expression	Tensor signature
Deep gravity well — REL axis deformation	Attachment seeking behaviours that paradoxically recreate unsafe dynamics. The well curves toward familiar relational patterns regardless of their harm.	REL score chronically low. High CRPF. Tethering volatility detectable in REL time-series.
Flattened convergence surface — high Ω	Reduced response to standard therapeutic intervention. High action (S) required to traverse the transition zone. Treatment appears 'not working'.	$\Phi(t)$ remains elevated despite intervention. $\delta(t)$ flat. κ compressed.
Brainstem-dominant curvature — SOM deformation	Somatic hypervigilance, freeze responses, sensory overwhelm triggering before cortical processing is online. Body is tracking threat the conscious mind has not yet registered.	SOM axis elevated baseline. EREG volatility. NMT Tier 1 redirection flagged.
Identity gravity well — VIC/NI deformation	Narrative lock. The story of self has become a gravity well — all new evidence bends toward confirming the existing narrative rather than updating it.	VIC score high and stable (low volatility = narrative rigidity). NI fragmented.

12.3 The Counter-Vector — Modulating the Geometry

Therapeutic intervention does not remove the deformation. The mass that created the curvature — the actual events — cannot be undone. What changes is the geometry's influence on current geodesics. The counter-phase regulatory wave $R(t)$ introduces an opposing curvature — not erasing the gravity well but reshaping the geodesic field around it so the individual has access to paths that bypass it.

EXTENDED — 2026:

The effectiveness of any intervention is a function of where it is applied on the manifold and whether it addresses the deformation at the correct NMT tier:

$$R(t) = A^R \times \cos(2\pi ft + \phi^R)$$

Applied at the correct tier, frequency, and phase, $R(t)$ achieves destructive interference with the pathological geodesic attractor — reducing its pull. Applied at the wrong tier (e.g., cognitive reframing applied to a brainstem-level deformation), $R(t)$ produces no change in the underlying geometry. The geodesic is not altered by talking about it from above. This is the mathematical encoding of Perry's neurosequential principle: brainstem first. The geometry must be addressed from the base upward.

12.4 Environmental Context $E(t)$ — Curvature Modifier

The environmental context tensor $E(t)$ does not produce trauma residual directly — but it modulates the curvature of the manifold in real time. A high-stress, low-resource environment increases the depth of existing gravity wells. A safe, supported environment reduces their pull without erasing them.

$E(+)$ → geodesic field expansion → more paths available
 $E(-)$ → geodesic field contraction → fewer paths available

This is why environmental improvement alone does not resolve trauma (Ω remains unchanged) but why environmental safety is a necessary precondition for therapeutic work — it expands the geodesic field sufficiently for the individual to access the interventions that can reduce Ω over time.

Clinical implication:

You cannot effectively reduce Ω in an environment that is actively re-traumatising. Environmental stabilisation is a prerequisite, not an alternative to trauma processing. The framework quantifies this as $E(t)$ modulation of the effective convergence surface — lower $E(t)$ load allows κ to function normally; high $E(t)$ load compresses κ and deepens the Ω deformation even when Ω itself is not actively increasing.

13 Part X: Theoretical Lineage and Strategic Alignment

Thought Leader / Framework	Connection to KINTSUGI COLLECTIVE / OMEGA RTI
Gabor Maté	Internal disconnection as the root of addiction and chronic illness. OMEGA RTI measures the magnitude and trajectory of that disconnection across time.
Judith Herman	Three-phase trauma recovery model maps directly to crisis, integration, and stabilisation thresholds.

Bruce Perry	Neurosequential Model of Therapeutics is the direct basis for the NMT tier architecture and intervention redirection logic.
Tim Fletcher	Internal parts model and identity diffusion concept mathematically represented through ID component of CRPF and NI axis.
Bessel van der Kolk	Body as primary trauma storage. Direct basis for SOM axis prioritisation and requirement to address somatic regulation before cognitive intervention.
Eckhart Tolle	Present-moment awareness as regulatory foundation. Observer self as the space between thoughts — represented in companion's presence-mode architecture.
Thich Nhat Hanh	Embodied mindfulness practice. Directly referenced in Sanctuary module design.
Andrew Huberman / Anna Lembke	Somatic balance, addiction cycles, physiological baseline regulation. Integrated into B(t) and I(t).
Jordan Peterson	Chaos and order as measurable states. Drift and return encoded into tensor shape and convergence geometry.
Albert Einstein / General Relativity	Spacetime curvature as the conceptual foundation for trauma as topological deformation.

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"What is your signal geometry?"

Not: 'What happened to you?'

KINTSUGI COLLECTIVE and RTI represent a paradigm shift in how the human organism is understood, monitored, and supported. The framework does not pathologise. It does not diagnose. It does not collapse the complexity of a human life into a DSM code or a six-session protocol.

It maps the field. It detects the perturbations. It applies counter-vectors where needed. It holds space where nothing else is needed. And it does all of this in service of one thing:

Helping people regain navigation rights to their own lives.

Circumstances don't define you, they reveal who you are.

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